A PRIVATE BRIEFING OVER LUNCH, EL PASO, THURSDAY, NOVEMBER 5, 2026

Six months of free decisions.

What to automate, what to buy, and what to leave alone before the new center opens.

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WHERE YOU ARE

The MRI is signed. No staff hired, no systems chosen.

Today

Every operational decision is cheap and reversible.

2027-01-01

Federal prior authorization APIs go live at the payers.

~2027-03

The build starts, so prior authorization is working before you open.

~2027-06

Target opening for your new center, April at the earliest. Whatever is in the vendor contracts by then is what you live with.

Greenfield is the entire argument. Nothing on the slides that follow is possible to do this cheaply after opening.

CMS-0057-F, 89 FR 8758, compliance dates verified in the Federal Register rule text.

THE FREE MOVE

Put four things in every vendor contract before you sign. They cost nothing today.

- **API access** to your own data, named in the contract, not "available on request."
- **HL7 interface count and fee schedule** fixed at signature, not quoted per interface later.
- **Data export rights** in standard formats, on demand, at no charge.
- **DICOM query permissions** (worklist, query/retrieve, DICOMweb) for systems you authorize.

Before signature these are contract terms. After signature they are hostage terms.

THE HEADLINE NUMBER

11.5%

of denied prior authorizations were appealed

80.7%

of those appeals were partially or fully overturned

Four in five appeals win. Almost nobody files one.

Source: KFF, "Medicare Advantage Insurers Made Nearly 53 Million Prior Authorization Determinations in 2024," 2026-01-28, analyzing data insurers report to CMS. Medicare Advantage, all services; CMS does not report prior authorization data by service type.

WHY GREENFIELD MATTERS

40%

of physicians report having staff who work **exclusively** on prior authorization.

You have not hired that person yet.

Source: American Medical Association, 2025 AMA Prior Authorization Physician Survey, 1,000 practicing physicians (400 primary care, 600 specialists). Self-reported survey data.

BEFORE THE NUMBERS: WHAT AN AI AGENT IS

Software that does a job inside your systems, and asks a person before anything leaves the building.

One that runs a workflow

An operations agent works one process from start to finish: a prior authorization, a cancelled slot filled from the waitlist, a records request. It works through your RIS and the payers' own electronic connections, not by clicking around a screen. Someone on your staff approves anything that goes out.

One you talk to

An owner's agent you text or call. Ask how many MRIs were read last week, or which referrer went quiet, and it answers from your own numbers. Every Monday it sends a short report: volumes by modality, reader, and referrer; authorization and denial rates; reconciliation.

The guardrails

- **A person signs** anything clinical and anything that leaves the building.
- **Numbers are calculated and checked**, never guessed. If a figure cannot be verified, it says so.
- **HIPAA business associate agreements** with every vendor that touches patient data.
- **Patient data stays in the United States.**

PART ONE

The thing you did not ask about.

Prior authorization and eligibility verification. Scheduling, phones, and records are real costs. This one is larger, and it is the one with the best evidence.

WHAT PRIOR AUTH COSTS YOU

13

hours a week per physician on prior authorization. Each request worked by hand.

95%

get approved anyway.

A process that approves 95% of what it touches is one you automate, not one you staff.

Sources: American Medical Association, 2025 AMA Prior Authorization Physician Survey; self-reported. Harrer S et al., J Am Acad Orthop Surg, 2025 Feb 15;33(4):e244-e252, PMID 39729599; single institution, outpatient MRI, 2019 through 2022.

WHY NOBODY APPEALS

52%

of physicians who do not appeal cite staff time.

59%

do not expect to win.

Both are what an agent removes.

Source: American Medical Association, 2025 AMA Prior Authorization Physician Survey; self-reported.

THE DATES LAND ON YOUR OPENING

2026-01-01

Payers must decide prior authorizations in **7 calendar days** standard, **72 hours** expedited, and publish their metrics.

2027-01-01

Payers must operate a **FHIR Prior Authorization API**, plus Patient Access, Provider Access, and Payer-to-Payer APIs.

Target ~2027-06

Your new center opens into the new regime, with prior authorization already working and no legacy process to unwind.

Who the rule reaches: Medicare Advantage, Medicaid, CHIP and exchange plans. Not employer-sponsored plans, TRICARE, or VA. The payers who move first set the standard the rest follow.

Almost 63% of medical plans report they are already building a FHIR prior authorization API ahead of the deadline.

Sources: CMS-0057-F, 89 FR 8758, compliance dates verified in rule text; covers Medicare Advantage, Medicaid and CHIP, and federally facilitated exchange plans. 63% figure: 2025 CAQH Index (DataSpring), 2026-02-19, payer self-report.

ONE ORDER THROUGH PRIOR AUTHORIZATION

An MRI order arrives at 9:00 on a Tuesday. Here is what the agent does with it.

1. The order comes in from the referring office and lands in your RIS.
2. The agent checks the patient's coverage and whether this payer requires authorization for this study.
3. If it does, the agent assembles the request from the order and the clinical notes, and checks it against that payer's rules.
4. Your staff member reviews it and approves. Only then does it go to the payer, over the payer's electronic connection.
5. The agent watches the payer's decision clock and puts the approval on the schedule the moment it arrives.
6. If it is denied, the agent drafts the appeal with the reasons and puts it in front of a person. Nothing goes out unsigned.

You will click through this yourself: a working prior authorization demo on synthetic data, no real patients, already built. The real one is wired to your RIS and payers in Phase 4.

rfinlay.github.io/imaging-priorauth-demo

Scan to follow along

PART TWO

The three things you did ask about, corrected.

Scheduling. Answering patients. Requesting records. All three are real. Two of them are not what they look like.

SCHEDULING: THE EMPTY SLOT IS NOT A NO-SHOW

22.3%

of outpatient radiology exams were cancelled

2.0%

were no-shows

70.2% of cancellations were patient-initiated, which means most empty slots are known in advance. That is a backfill problem, not a reminder problem.

Source: Aijaz A et al., "Sociodemographic Factors Associated with Outpatient Radiology No-shows Versus Cancellations," Acad Radiol, 2024 Aug;31(8):3406-3414, PMID 38705764. University of California, Irvine. 19,262 exams, 9,713 patients. Single center.

SCHEDULING: WHAT REMINDERS HONESTLY DO

5.1%

MRI no-show rate with phone reminders alone

3.8%

after adding a text message reminder

A real effect, 1.3 percentage points, in a randomized trial of 6,989 MRI patients. It did not improve arrival punctuality. **Your RIS will do this. Let it.**

Source: Liu C et al., "Text Message Reminders Reduce Outpatient Radiology No-Shows But Do Not Improve Arrival Punctuality," J Am Coll Radiol, 2017 Aug;14(8):1049-1054, PMID 28583321. Massachusetts General Hospital. p = 0.02; only 67.5% of the texting arm could be reached by text.

ANSWERING PATIENTS: BUY IT, DO NOT BUILD IT

26.5%

of El Paso County residents aged 5 and over speak English less than "very well." 97.8% of them speak Spanish.

My recommendation: buy a healthcare voice vendor. I will write the specification, run the evaluation, and integrate and supervise it. Voice is the highest-blast-radius failure in this whole list, and it is a funded category I cannot beat on cost.

Source: American Community Survey 2024 5-year, Table B16004, El Paso County, population 5 and over of 813,194; 215,186 speak English less than "very well." Computed from raw ACS counts via Census Reporter.

The 30-day clock runs from receipt

Not from when the request reaches the right desk. One extension of up to 30 more days, with written notice inside the first 30. Outer limit 60 days.

Finding the study is unbillable

Fees may cover labor to copy, supplies, postage, and an agreed summary. Search and retrieval labor is explicitly not chargeable. The expensive half of the work cannot be charged for.

The harder problem is outbound. A new center opens with zero priors on file. Every comparison study comes from somewhere else, and RSNA is on record that DICOM exchange is not yet a TEFCA requirement.

Sources: 45 CFR 164.524 (right of access, deadlines, permitted fee components); Ciox Health v. Azar, 435 F. Supp. 3d 30 (D.D.C. 2020), which left the $6.50 flat-fee option in place; RSNA comment letter to ASTP/ONC, March 2026. OCR's Right of Access Initiative has produced more than 50 enforcement actions.

THE FIFTH CANDIDATE: THE NUMBERS THE OWNER ACTUALLY LOOKS AT

Studies by modality, by reader, by referrer. Turnaround. Scheduled versus completed. Revenue per slot.

All of it already lives in the RIS and the billing system. The problem is that they are two systems, and RIS reporting is built for radiology operations, not for an owner.

As an owner, I want to ask one question on a Monday morning, which referrer went quiet, what is the appeal rate, are we being paid for what we read, and get the answer from my own numbers, **so that** I run the center on facts and not on a report someone has to build for me. The manager and the front desk lead see the same numbers from their side: today's schedule, the open authorizations, the slots that just opened. Smallest compliance envelope of the five. Cheapest to build.

THE SCORECARD

CANDIDATE	MY RECOMMENDATION	WHAT IT DOES
Prior authorization and eligibility	**Build**	Spots the orders that need authorization, checks coverage, assembles the packet, submits through the payer's API or EDI, watches the decision clock, drafts the appeal. A person approves every submission.
Scheduling: reminders and self-scheduling	**Buy (the RIS)**	Text and call reminders, online booking, intake forms. Your RIS's own feature.
Scheduling: cancellation backfill	**Build, small**	When a slot opens, offers it by text to a ranked waitlist, inside the per-patient message cap, and books the first yes.
Answering patients (voice)	**Buy, I integrate**	A vendor answers and routes patient calls in English and Spanish. I connect it to the schedule and supervise it.
Records: inbound requests	**Build, selectively**	Intake, an identity check step, the statutory deadline clock (15 business days in Texas), fee calculation, status, DICOM export of the study.
Records: outbound priors	**Build, selectively**	Tracks requests to other facilities, follows up, imports what arrives, flags the radiologist when a prior is still missing.
Owner reporting	**Build**	Joins the RIS and billing data. A Monday report, and an agent you ask questions of in plain words.
Billing, claims, and the eligibility feed	**Buy**	The clearinghouse and the claims work, on your billing vendor's system. I evaluate reachability and contract terms. Row 1 reads the eligibility feed; it does not replace the vendor.

PART THREE

Proof: RadAssist, in daily use.

Structured radiology reporting, in production and in daily use by the radiologist sitting at this table. His template, his rules, his signature. It drafts; he signs. Nothing you type leaves the browser.

Calcium scoring

Reads the scanner's numbers from a pasted screen capture. He confirms every value before it applies.

Thyroid ultrasound

ACR TI-RADS, with his follow-up rules built in.

Spine MRI, new

Cervical, thoracic, and lumbar. Built from Dr. Ramos's own template and rules. Every rule in it is his.

CT Spine, live Nov 2

The same tool, switched to CT: bony canal, calcification, the CT disc findings. Built from his sample reports two days after the MRI module.

Next, at his request: voice dictation, brain, cardiac CTA (CAD-RADS). Reading the images themselves is a future item with a regulatory path, not a promise.

radassist.app · Not a device: it formats what the radiologist graded. It never reads the image. At your center, the signed report flows into the RIS over HL7 v2 ORU, a contract term from slide 3.

> Software intended "for administrative support of a health care facility, including the processing and maintenance of financial records, claims or billing information, appointment schedules, business analytics, information about patient populations, admissions, practice and inventory management, ... determination of health benefit eligibility, population health management, and laboratory workflow" is not a medical device.

Every candidate on the scorecard is on that list by name. The agents stop at the clinical boundary: nothing interprets an image, selects a protocol, or recommends a study.

Source: 21 U.S.C. 360j(o)(1)(A), read verbatim at Cornell Legal Information Institute. 360j(o)(2) further provides that FDA "shall not regulate" excluded administrative functions even when they coexist with device functions.

Texas SB 1188

Electronic health records with Texas patient data must be physically stored in the United States as of 2026-01-01. Every component, including the model provider, gets verified for US residency before anything is promised.

TCPA, wireless healthcare exemption

One message per day, three per week, per patient. No billing or financial content. Provider name and opt-out in every message. $500 per violation, up to $1,500 if willful.

The AI vendor's BAA

Anthropic's agreement now covers Skills, Web Search, Memory, and the Bash and Text Editor tools. It still excludes Computer Use and Web Fetch, which are exactly what you would reach for to drive a payer portal with no API. So the prior auth design routes through APIs, EDI, or a human step. Not screen-driving.

Texas HB 300

Texas privacy law covers me directly, not just through our contract. If I mishandle your patients' data, I'm the one the state comes after. That's how it should be.

Sources: Texas SB 1188 (89R), signed 2025-06-20, effective 2025-09-01, bill history at capitol.texas.gov; 47 CFR 64.1200(a)(9)(iv) and 47 U.S.C. 227; Anthropic BAA documentation at privacy.claude.com, accessed 2026-10-01; Texas Health and Safety Code Chapter 181.

THE QUESTION YOU ARE GOING TO ASK

What happens when I get hit by a bus?

- **You own the specification.** It is written for a competent engineer who has never met me, and it is the deliverable of the design engagement whether or not I build anything.
- **You own the code.** Everything specific to this center lives in a repository under your organization's account, with my access revocable by you. What you are buying is a scope of work: a design for one imaging center that can be put anywhere. Who owns any product pieces is settled in the contract, and you are never hostage to me.
- **Runbooks, not memory.** Every operational procedure is written down: how it runs, how it fails, who to call at each vendor.
- **No proprietary runtime.** Standard interfaces (HL7 v2, DICOM, payer APIs) and a named model vendor under its own BAA. Any integrator can take it over.
- **Buy where buying is right.** The voice vendor and the RIS are yours directly. Nothing routes through me that does not have to.

THE MOST IMPORTANT SLIDE

What I recommend you do not buy from me.

- **A custom voice agent.** Buy a vendor. I will specify, evaluate, and integrate it, and I will tell you when it is not good enough in Spanish.
- **Reminders, self-scheduling, and intake forms.** Your RIS does this. Make sure the contract says so, then use it.
- **Anything built to resell to other centers.** This center is the reference implementation and the priority. If I ever take on a second site, your service terms do not change, in writing.
- **Anything that touches the image clinically.** Not from me, not from anyone, without an FDA pathway.

IF IT WERE MY MONEY

I would do three things before signing another vendor contract.

- Put the four access terms into every contract this month. It is free.
- Decide now whether the prior authorization desk is a hire or a system. That decision expires the day you hire for it.
- Tell me what front desk and prior authorization actually cost you last time. Your number beats any benchmark I could bring.

And one question for you: what would you do differently this time, building it from scratch?

WHAT THE DATES DEPEND ON

The build starts in March so everything works before you open. These have to be true first.

- **A signed agreement.**
- **Your vendors selected and contracted** (Phase 2), one pick per system, with sandbox access to their interfaces: API, HL7, and DICOM.
- **Vendor contracts and HIPAA business associate agreements signed** before the build starts (Phase 4), and the hosting option chosen by the end of Phase 3.
- **Test access to the payers' connections** for prior authorization, FHIR and EDI, and payer enrollment and credentialing started early enough to bill at opening.
- **One named person on your side** who approves.
- **Your expected volumes and payer mix**, before payer-specific design starts.
- **Equipment delivery dates**, so the build lands on the install.

If one of these slips, the dates move with it, and the price does not. You hear about it the day it happens.

WHAT I AM PROPOSING

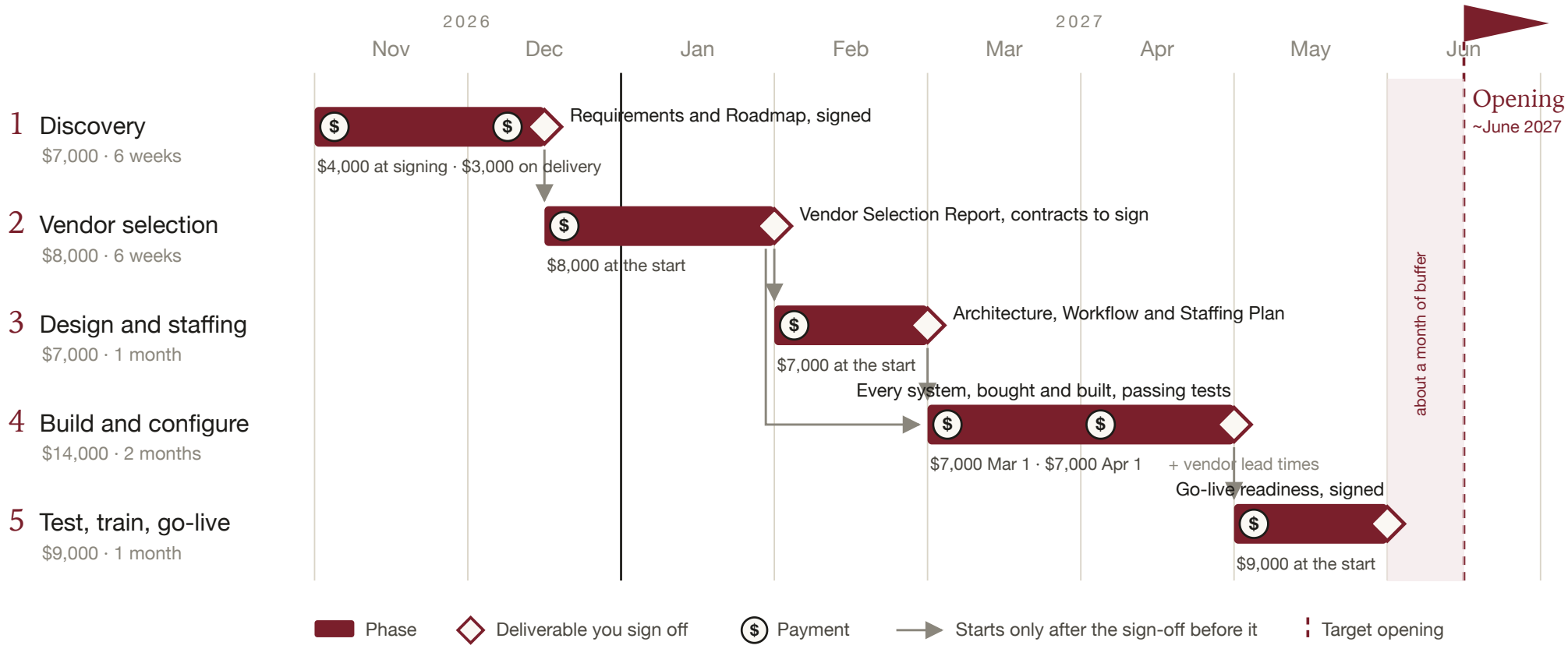
One engagement, five phases, forty-five thousand. Each phase ends in a sign-off.

#	PHASE	WHAT YOU SIGN OFF	WHEN	FEE	PAID
1	**Discovery and requirements** How the new center runs: patient flow, services, volumes, payers, what is automated at opening	Requirements and Roadmap, signed Requirements per system; patient-flow map, order to signed report to paid claim; contract terms; make-or-buy for each requirement: a system you buy, or a piece I build, with a ballpark running cost for anything built; shortlist and criteria for Phase 2	Nov to mid-Dec 6 weeks	**$7,000**	$4,000 at signing $3,000 on delivery
2	**Vendor and system selection** Scheduling; RIS and PACS, bundled cloud system or separate RIS plus PACS (Exa is one example); prior authorization and eligibility; patient phones, IVR and AI answering; radiologist dictation; billing and clearinghouse; modality worklist, reading workstations, image sharing	Vendor Selection Report, plus the list of contracts to sign, interface fees fixed up front Side-by-side comparison; weighted scoring against the Phase 1 requirements, integration readiness a named criterion; pros and concerns; 5-year total cost per option; a recommendation	Mid-Dec to Jan 6 weeks	**$8,000**	In full at the start
3	**Design and staffing plan** Systems, network, workflows, security and HIPAA, how each role's work runs	Architecture, Workflow and Staffing Plan, plus Total cost of ownership and operating budget Three hosting options, three years, you pick	Feb 2027 1 month	**$7,000**	In full at the start
4	**Build and configure** Systems set up and connected, RadAssist configured, prior authorization automated	Every system, bought and built, set up, connected and passing end-to-end tests Vendors configure their own products; I manage them, connect everything, run the tests	Mar to Apr 2027 2 months	**$14,000**	$7,000 on March 1 $7,000 on April 1
5	**Testing, training and go-live** Staff trained on the real systems; opening week on call	Signed go-live readiness, plus opening-week support	May 2027 1 month	**$9,000**	In full at the start
	Fixed fee, fixed scope	**Ready for a June 2027 opening, about a month of buffer**	**Nov to May**	**$45,000**	
	After opening: support plan	Monitoring, fixes, updates to what I built, vendor coordination, a response-time promise; up to 20 hours a month, $150 an hour beyond	From June 2027, 12 months, 90-day check-in	**$3,000/mo**	Separate from the $45,000

Priced for one center, with one selected system per category; additional centers, or a different system for another center, are quoted and approved before work starts. Running costs after the build (cloud, AI usage, texting, clearinghouse, security) are not in the $45,000: billed to the center at cost in accounts in your name, estimated in Phase 1, budgeted in Phase 3, monthly statement (slide 32). If the opening date moves, the timeline moves with it; the price does not. Work outside these phases is approved first and billed at $150 an hour against a retainer you top up.

THE SEVEN MONTHS, ON ONE PAGE

Five phases, five sign-offs, one June opening.



If the opening date moves, every bar moves with it. The build waits on your vendors' lead times as well as on Phase 3, which is why vendor selection comes early.

WHAT I NEED FROM YOU

Each phase runs on a few things only you can give. None of them is a surprise.

#	PHASE	WHAT I NEED FROM YOU	WHAT YOU GET BACK
1	Discovery and requirements	Time with all three of you: Dr. Ramos, Fernando and Jesse. The services and modalities you plan to offer, expected volumes and payer mix, and the target opening date. Dr. Ramos's current center is an input, not the template.	Requirements and Roadmap, signed by you: requirements per system, the patient-flow map (order to signed report to paid claim), contract terms, a make-or-buy recommendation for each requirement (a system you buy, or a piece I build, with a ballpark running cost for anything built), shortlist and criteria for Phase 2
2	Vendor and system selection	Budget ranges, any vendors you already prefer, your radiologists' dictation preference, and your decisions as the scoring comes in.	Vendor Selection Report: side-by-side comparison of the shortlisted systems in each category, weighted scoring against the Phase 1 requirements, integration readiness as a named criterion, pros and concerns, 5-year total cost of ownership per option (license or subscription, implementation, interface fees, support, hardware), a recommendation; plus the contracts to sign for every system on slide 31
3	Design and staffing plan	Operating hours, the roles you intend to hire, the vendor contracts as they are negotiated, and your hosting pick from the three options.	Architecture, Workflow and Staffing Plan, plus Total cost of ownership and operating budget (three hosting options, three years)
4	Build and configure	Signed vendor contracts and business associate agreements, sandbox access to each system, and equipment delivery dates.	Every system, bought and built, set up, connected and passing end-to-end tests. Vendors configure their own products; I manage them, connect everything and run the tests
5	Testing, training and go-live	Your staff, hired and available for training, and the final opening date.	Signed go-live readiness, and me on call through opening week

One named person on your side who approves. Jesse, I would especially want your time on the technical and integration decisions. Everything I produce is written for an engineer who has never met me, and you own all of it.

THANK YOU

Every one of these decisions is reversible today. None of them will be in six months.

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APPENDIX

Reference, only if asked.

Nothing here is part of the pitch. It is what stands behind the slides if the room wants to go one level deeper, today or another day.

In this appendix: the systems to select in Phase 2, with a starting shortlist; what it costs to keep it running, and the three ways to host it; the opening checklist; ideas worth a look (two slides); radiology report assistance.

SYSTEMS TO SELECT IN PHASE 2: STARTING SHORTLIST

A starting shortlist, not a decision. The final pick is Phase 2, with your radiologists in the demos.

SYSTEM	STARTING SHORTLIST	WHAT DECIDES IT
RIS and PACS	Evaluated both ways: one bundled cloud system, or separate best-of-breed RIS plus PACS. Bundled candidates built for independent centers: Konica Minolta Exa PACS|RIS, RamSoft OmegaAI, Novarad, Intelerad. The hospital-sized award winners (Sectra, Agfa) are built for a different buyer; a premium viewer with a separate RIS only if the radiologists insist.	A reading demo with your radiologists on MRI and CT cases; five-year cost with interfaces, storage and dictation; front desk to claim in one workflow; hosting and security (BAA, SOC 2, backup, uptime); export and API terms; two or three single-site references; a timeline before June.
Dictation	The radiologists choose the tool, you choose the contract: the RIS's built-in dictation, PowerScribe One (Microsoft), Fluency for Imaging (Jacobian; 2026 Best in KLAS), Rad AI. A contracted group may already carry a license.	What your radiologists already use; the interface fee for the link to the RIS and PACS; per-radiologist cost at one to three readers.
Billing and clearinghouse	Outsource billing to an imaging-specialized firm at opening, short contract, clearinghouse and data in your name. Clearinghouses in common use: Waystar, Availity, TriZetto, Office Ally, Quadax (2026 Best in KLAS). A documented second path.	Three quotes; TRICARE, Medicare and Medicare Advantage handling; eligibility and authorization transactions reachable by the Phase 4 build; handover terms.
Also selected in Phase 2	Scheduling and reminders (inside the RIS); prior authorization and eligibility connections; patient phones, IVR and AI answering (patient-facing, separate from dictation); modality integration and worklist; reading workstations; image sharing (CD and cloud). Each carries the four contract terms from slide 3; interface fees fixed up front. Every system is scored on how easily it connects to the rest, and the integration terms go into the contract.	

Basis: 2026 Best in KLAS awards (klasresearch.com, Feb 2026); vendor documentation for Konica Minolta, RamSoft, Microsoft PowerScribe One, Jacobian; no vendor on this slide has been evaluated or quoted. Fit and pricing are confirmed in Phase 2.

WHAT IT COSTS TO KEEP RUNNING, AND WHERE IT RUNS

Running costs are not in the $45,000. They run in your accounts, at cost, with a monthly statement.

	A. YOUR SECURE CLOUD ACCOUNT	B. HYBRID: A BOX AT THE CENTER, PLUS YOUR CLOUD	C. ALL ON-SITE, INCLUDING A LOCAL AI MODEL
Patient data and the AI	Goes to the AI company under a signed HIPAA agreement, never kept, never trained on	Names and IDs are swapped out here first; clinical details still go under the agreement	No AI company sees it. Payers, the clearinghouse and backups still do
If the power or internet goes out	Keeps working	Pauses, then catches up	Stops
AI quality	Best available	Best available, same model	Lower; has to be proven on your cases first
Your staff's extra duties	None	Lock the room, keep a log	Lock the room, keep a log, an outage plan
Vendors, per month, estimated	About $900 to $4,200; expected near $2,000	About $1,000 to $4,300; expected near $2,100	About $1,200 to $3,500; expected near $2,000, plus the hardware
One-time setup, estimated	$6,000 to $25,000	$12,000 to $37,000	$27,000 to $52,000
Support plan, from June 2027	$3,000 a month, up to 20 hours; $150 an hour beyond	Same, plus upkeep of the box and the swap step (extra hours)	Same, plus model upkeep and re-testing on every change (extra hours)
Expected monthly, vendors plus support	About $5,000	About $5,100, plus extra hours	About $5,000, plus hardware and extra hours
Where I lean today	**A, with B's name-swapping step inside your cloud account if you want it**	Only if your RIS forces a local connection	Not for opening; a measured test later

Every account is in your name and you sign its HIPAA agreement; I hold the admin keys, you hold the ownership, nothing is marked up. Ranges are estimates at about 650 studies a month, confirmed in Phase 1, firmed into an operating budget in Phase 3, with a cap I cannot pass without your written OK.

OPENING CHECKLIST: REQUIRED TO OPEN, OUTSIDE THE FEE

Six things the center must do to open and bill. None is my build. Each gets a named owner in Phase 1.

ITEM	WHY IT MATTERS	OWNER	WHEN
Payer enrollment and radiologist credentialing	Medicare IDTF enrollment, Texas Medicaid, TRICARE, commercial payers, CAQH. No enrollment, no revenue; the longest lead item on this list.	Center administrator, with a credentialing service	Starts in Phase 1
ACR accreditation, MRI and CT	Medicare requires it to bill the technical component. Brings CT dose records, an MR safety policy and roles, physician peer review and a physicist QC program with it.	Medical director, with a contracted physicist	Plan in Phase 1, submit after go-live
Texas DSHS CT registration	Registration, shielding plan, dosimetry, operator credentials. The CT cannot run without it; an inspection item.	Physicist and center administrator	Starts in Phase 1
CLIA certificate of waiver	Needed before the first fingerstick creatinine for contrast screening.	Center administrator	Before opening
HIPAA security basics	Risk analysis, MFA and endpoint protection, backup and downtime plan, incident response and Texas breach notice.	Designed in Phase 3; the center owns it	Phase 3 to 5
HB 300 staff training	Within 90 days of hire, signed statements kept; plus the posted records-request notice.	Center administrator	As staff are hired

Basis: 42 CFR 410.33 and 414.68; 25 TAC 289.226, 289.227, 289.202; 42 CFR 493; 45 CFR 164.308 to 164.316; Texas HSC 181.101. Owner roles are proposed; the names go in the Phase 1 Requirements and Roadmap.

IDEAS WORTH A LOOK: NOT IN THE FEE, SCOPED ONLY IF YOU ASK

When your radiologist says "repeat in six months," we make sure it happens.

Follow-up recommendation tracking

Every report that recommends more imaging is logged, the referrer is told, and at the due date someone checks it happened. Tracked programs completed follow-up 78% of the time versus 52% untracked; one private radiology group's own tool took lung-nodule follow-up from 26.5% to 59.7%. Hospital settings; outreach goes through the referrer, never a direct solicitation. A small build on top of report delivery.

Good-faith cost estimates at scheduling

Across 470,422 outpatient imaging visits at UC Irvine, patients given a staff-generated out-of-pocket estimate had nearly twice the odds of completing the exam (OR 1.91). An association, not a trial. Rides on the eligibility feed and satisfies the No Surprises Act in the same step.

Sources: Respir Med 2022, PMID 35051877 (78% vs 52%, nurse-navigator program); BMJ Open Qual 2019, PMID 31206047 (26.5% to 59.7%); Rashidi et al., Clin Imaging 2025, PMID 39736182 (OR 1.91, 95% CI 1.87 to 1.95). Academic and hospital settings; transfer to a single outpatient center is not measured.

Three smaller ones. Each is bought or configured, not built.

- **Structured referrer orders.** At Emory only 51% of abdominal CT requisitions carried the patient's cancer history. If the order arrives complete, prior auth is mostly paperwork; if it does not, it is phone calls. Mostly a RIS referrer-portal feature; the field list is Phase 3 work.
- **Peer learning, not score-based review.** Accreditation requires peer review anyway. Set it up as peer learning, which took radiologists less than half the time in one study and produced far more useful feedback. A PACS module or the ACR program; nothing to build.
- **Contrast pre-screening plus fingerstick creatinine.** The kidney questions go into the pre-visit intake; a patient who screens positive without a recent result gets a two-minute fingerstick on arrival, not a second appointment. Needs the CLIA waiver on the opening checklist.

Sources: Driessen et al., Curr Probl Diagn Radiol 2026, PMID 40517119 (51% of requisitions; the link to denials is inference, not a measured number); Kwak et al., Clin Imaging 2024, PMID 38113549 (0.76 vs 1.71 hours a month per radiologist); point-of-care creatinine cost-effectiveness, PMID 34339953. None of these is in the $45,000.

APPENDIX: RADIOLOGY REPORT ASSISTANCE, THE TWO DIRECTIONS

The agent drafts. The radiologist signs. Nothing clinical leaves without a human signature.

The report: RadAssist, today

Dr. Ramos uses it daily. Calcium scoring reads the scanner's numbers from a screen capture; thyroid, spine MRI, and CT spine build the report from his own template, one dropdown per sentence, with his rules for the impression. He confirms every value before it applies and signs. At your center the path has a name: DICOM worklist and query for the study, HL7 v2 ORU to carry the signed report into the RIS.

The records: a different build

Requesting records runs the other way: DICOM query and retrieve where a facility grants it, a request out and DICOM import where it does not, and the payer Provider Access API in 2027 for the claims record, in-network only, no images.

Which did you mean?

Two different builds, specified separately. I would rather ask than guess.

Across centers: one environment, isolated instances. Never one shared database.

One specification, reused for every center. The default is the industry standard, security first: one environment under one contract, and each organization in its own isolated instance, with its own database and encryption keys, its own credentials and role-based access, and its own audit log. No organization sees another's data. One owner or one joint venture: that is the whole answer. Separately owned centers that want fully separate hosting: a fully separate stack each, at higher cost. Either works.